NCCP 00619 · VERSION 4A

Adjuvant abemaciclib + endocrine therapy

The usual abemaciclib starting dose is 150 mg twice daily; a lower dose or altered schedule may be prescribed. Abemaciclib is taken twice daily continuously for up to two years with endocrine therapy, unless recurrence or unacceptable toxicity requires an earlier change. A four-week dispensing block is not a rest cycle. Endocrine treatment normally continues beyond abemaciclib.

Make treatment fit into daily life

Take abemaciclib twice daily at regular times with or without food. Swallow whole; do not split, chew or crush. Avoid grapefruit. Do not replace a missed or vomited dose. A clinical hold and restart require the treating team’s instructions.

Before starting

Confirm the exact medicine, strength, endocrine partner, baseline tests and first review appointments. Ask who arranges tests and who reviews the results.

Taking treatment

Take abemaciclib twice daily at regular times with or without food. Swallow whole; do not split, chew or crush. Avoid grapefruit. Do not replace a missed or vomited dose. A clinical hold and restart require the treating team’s instructions.

Treatment duration

At completion of the planned two-year abemaciclib course, continue the separately agreed endocrine and follow-up plan.

Supply and appointments

Request the next supply before tablets run out. Blood tests and any ECG or injections need their own appointments. This oral treatment does not itself require a central line or infusion pump.

If treatment is held

Ask which medicine is held, whether the endocrine partner continues, what needs to recover, when the next review is due and who will authorise restart.

Support between appointments

Keep the oncology contact details and prescribed symptom medicines accessible. New severe symptoms need prompt assessment rather than waiting for the next blood test.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00619 v4a · official regimen
- EMA · Verzenios product information and patient leaflet
- NICE NG101 · bone health and follow-up
- ESMO 2024 early breast cancer guideline · treatment and follow-up
- ESC 2022 cardio-oncology guideline · cardiac surveillance and risk
- HSE · chemotherapy side effects and urgent advice
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · PICC patient information
- eviQ · implanted port patient information
- monarchE: seven-year invasive disease-free survival · primary report
- EMA Verzenios current product information; checked 2026-10-25
- monarchE primary OS and seven-year outcomes, Johnston et al; online 2025, Annals of Oncology 2026

Source reconciled; independent clinical, pharmacy and patient/PPI review pending